

STATE OF NEW YORK

2719--A

2025-2026 Regular Sessions

IN ASSEMBLY

January 22, 2025

Introduced by M. of A. RAJKUMAR -- read once and referred to the Committee on Mental Health -- committee discharged, bill amended, ordered reprinted as amended and recommitted to said committee

AN ACT to amend the mental hygiene law, the social services law, the correction law, the judiciary law, the criminal procedure law, the public health law and the administrative code of the city of New York, in relation to enacting the "empire state of mind act" to address the treatment of persons with mental illness; to amend chapter 554 of the laws of 1986, amending the correction law and the penal law relating to providing for community treatment facilities and establishing the crime of absconding from the community treatment facility, in relation to the effectiveness thereof; and to repeal certain provisions of the correction law relating to certain limitations of community treatment facilities

The People of the State of New York, represented in Senate and Assembly, do enact as follows:

- 1 Section 1. This act shall be known and may be cited as the "empire
2 state of mind act".
- 3 § 2. Subdivisions 3 and 20 of section 1.03 of the mental hygiene law,
4 subdivision 3 as amended by chapter 281 of the laws of 2019, subdivision
5 20 as added by chapter 978 of the laws of 1977, are amended to read as
6 follows:
- 7 3. "Mental disability" means mental illness, intellectual disability,
8 or developmental disability~~[, or an addictive disorder as defined in~~
9 ~~this section]~~.
- 10 20. "Mental illness" means an affliction with a mental disease or
11 mental condition which is manifested by a disorder or disturbance in
12 behavior, feeling, thinking, or judgment to such an extent that the
13 person afflicted requires care, treatment and rehabilitation; or an
14 addictive disorder as defined in this section.

EXPLANATION--Matter in italics (underscored) is new; matter in brackets
[-] is old law to be omitted.

LBD06324-05-5

§ 3. Section 9.01 of the mental hygiene law, as amended by chapter 723 of the laws of 1989, the seventh undesignated paragraph as amended by chapter 595 of the laws of 2000, is amended to read as follows:

§ 9.01 Definitions.

As used in this article:

(a) "in need of care and treatment" means that a person has a mental illness for which in-patient care and treatment in a hospital or other facility, or receipt of services pursuant to this chapter or the social services law, is appropriate. Such services shall include, but are not limited to, employment, health care, mental health care, educational or vocational training, housing, and access to supportive outpatient facilities such as clubhouses.

(b) "in need of involuntary care and treatment" means that a person has a mental illness for which care and treatment as a patient in a hospital is essential to ~~[such person's welfare]~~ prevent such person from engaging in behavior likely to result in serious harm, and whose judgment is so impaired that ~~[he]~~ such person is unable to understand the need for such care and treatment.

(c) "likelihood to result in serious harm" or "likely to result in serious harm" means ~~[(a)]~~ 1. a substantial risk of physical harm to the person as manifested by threats of or attempts at suicide or serious bodily harm or other conduct demonstrating that the person is dangerous to [himself or herself] themselves, or [(b)] 2. a substantial risk of physical harm to other persons as manifested by [homicidal or other violent behavior by which others are placed in reasonable fear of serious physical harm] threats of, attempts at, or perpetration of violence, or possessing a weapon or an object that can be utilized as a weapon and using such weapon or object in a manner consistent with a use that would produce imminent physical harm to themselves or others; for the purposes of this section, noncompliance with instructions from any police officer, peace officer, or person engaged in homelessness intervention or behavioral health services in the absence of other threatening behavior shall not constitute reasonable fear of serious physical harm; nor shall the fashioning of any body part, or handling of any object that is not a firearm or component thereof in such a manner consistent with operating a firearm, constitute reasonable fear of serious physical harm.

(d) "need for retention" means that a person who has been admitted to a hospital pursuant to this article is in need of involuntary care and treatment in a hospital for a further period.

(e) "record" of a patient shall consist of admission, transfer or retention papers and orders, and accompanying data required by this article and by the regulations of the commissioner.

(f) "director of community services" means the director of community services for the mentally disabled appointed pursuant to article forty-one of this chapter.

(g) "qualified psychiatrist" means a physician licensed to practice medicine in New York state who: ~~[(a)]~~ 1. is a diplomate of the American board of psychiatry and neurology or is eligible to be certified by that board; or [(b)] 2. is certified by the American osteopathic board of neurology and psychiatry or is eligible to be certified by that board.

(h) "patient advocate" means an individual or organization with expertise in behavioral health and homelessness intervention services that provides assistance and representation to a patient receiving health-care, in support of such person's welfare.

(i) "empath unit" means a facility that provides emergency treatment to patients with mental illness without unnecessary long-term admission.

(j) "clubhouse" means a non-emergency medical facility and community-based psycho-social rehabilitation center that provides structured, evidence-based services for individuals living with mental illness, including:

(i) employment assistance, workforce development, and job placement;

(ii) housing assistance, supportive housing navigation, and homelessness prevention;

(iii) educational and skills training programs;

(iv) peer support groups and mental health counseling;

(v) crisis intervention and suicide prevention services;

(vi) re-entry support for justice-involved individuals; and

(vii) veteran services, including post-traumatic stress disorder support and United States department of veterans' affairs benefit navigation.

§ 4. Section 7.17 of the mental hygiene law is amended by adding a new subdivision (h) to read as follows:

(h) The commissioner shall establish at least one empath unit, as such term is defined in section 9.01 of this title, in each county in the state.

§ 5. Paragraph 2 of subdivision (a) of section 9.48 of the mental hygiene law, as added by chapter 408 of the laws of 1999, is amended to read as follows:

(2) The directors of assisted outpatient treatment programs shall ensure the timely delivery of services described in paragraph one of subdivision (a) of section 9.60 of this article pursuant to any court order issued under such section. Directors of assisted outpatient treatment programs shall immediately commence corrective action upon receiving notice from program coordinators, that services are not being provided in a timely manner. Such directors shall inform the program coordinator of such corrective action. Assertive community treatment team services which have been court ordered or otherwise directed under this article shall be provided within thirty days of such order or upon request of the application. For the purposes of such service provision, there shall be a presumption of eligibility to the maximum extent possible and confirmation of eligibility shall occur subsequent to program admission.

§ 6. Paragraph 2 of subdivision (b) of section 9.27 of the mental hygiene law, as amended by chapter 343 of the laws of 1985, is amended and a new paragraph 12 is added to read as follows:

2. the ~~[father or mother, husband or wife, brother or sister,]~~ parent or legal guardian, spouse, sibling or the child of any such person or the nearest available relative.

12. a behavioral health services expert acting as an agent of the city or county in which any such person may be.

§ 7. Section 9.27 of the mental hygiene law is amended by adding a new subdivision (j) to read as follows:

(j) A patient subject to an involuntary admission under this article shall be entitled to be placed in a room with a window and access to a restroom, and shall not be physically restrained unless such patient is determined to be dangerous.

§ 8. Subdivisions (a) and (d) of section 9.33 of the mental hygiene law, as amended by chapter 789 of the laws of 1985, are amended to read as follows:

(a) If the director shall determine that a patient admitted upon an application supported by medical certification, for whom there is no court order authorizing retention for a specified period, is in need of

1 retention or transfer to the jurisdiction of the department for
2 retention in a hospital operated by the state or to a private facility
3 having an appropriate operating certificate, and if such patient does
4 not agree to remain in such hospital as a voluntary patient or agree to
5 such transfer, the director shall apply to the supreme court or the
6 county court in the county where the hospital is located for an order
7 authorizing continued retention. Such application shall be made no later
8 than sixty days from the date of involuntary admission on application
9 supported by medical certification or thirty days from the date of an
10 order denying an application for patient's release pursuant to section
11 9.31, whichever is later; and the hospital is authorized to retain the
12 patient or transfer such patient to the jurisdiction of the department
13 for retention in a hospital operated by the state or a private facility
14 having an appropriate operating certificate for such further period
15 during which the hospital is authorized to make such application or
16 during which the application may be pending. The director shall cause
17 written notice of such application to be given the patient and a copy
18 thereof shall be given personally or by mail to the persons required by
19 this article to be served with notice of such patient's initial admis-
20 sion and to the mental hygiene legal service. Such notice shall state
21 that a hearing may be requested and that failure to make such a request
22 within five days, excluding Sunday and holidays, from the date that the
23 notice was given to the patient will permit the entry without a hearing
24 of an order authorizing retention or transfer.

25 (d) If the director of a hospital, in which a patient is retained
26 pursuant to the foregoing subdivisions of this section, shall determine
27 that the condition of such patient requires [~~his~~] further retention in a
28 hospital or transfer to a private facility having an appropriate operat-
29 ing certificate, [~~he~~] the director shall, if such patient does not agree
30 to remain in such hospital as a voluntary patient or does not agree to a
31 transfer, apply during the period of retention authorized by the last
32 order of the court to the supreme court or the county court in the coun-
33 ty where the hospital is located for an order authorizing further
34 continued retention or the transfer of such patient. The procedures for
35 obtaining any order pursuant to this subdivision shall be in accordance
36 with the provisions of the foregoing subdivisions of this section;
37 provided that the patient or anyone on [~~his~~] their behalf or the mental
38 hygiene legal service may request that the patient be brought personally
39 before the court, in which case the court shall not grant an order for
40 periods of one year or longer unless such patient shall have appeared
41 personally before the court. The period for continued retention pursuant
42 to the first order obtained under this subdivision shall authorize
43 further continued retention of the patient for not more than one year
44 from the date of the order. The period for the further continued
45 retention of the patient authorized by any subsequent order under this
46 subdivision shall be for periods not to exceed two years each from the
47 date of the order.

48 § 9. Section 9.03 of the mental hygiene law, as amended by chapter 351
49 of the laws of 2021, is amended to read as follows:

50 § 9.03 Admission to a hospital.

51 (a) Unless otherwise specifically provided for by statute, a person
52 with a mental illness shall be admitted to a hospital as an in-patient
53 only pursuant to the provisions of this article, except that chemically
54 dependent patients may be admitted to chemical dependence facilities
55 operated by such hospitals under contract or agreement with the office
56 of [~~alcoholism and substance abuse~~] addiction services and supports in

1 accordance with the provisions of article twenty-two of this chapter.
2 The section of the mental hygiene law under which a patient is admitted
3 or under which any change of legal status is subsequently effected shall
4 be stated in the patient's record.

5 (b) A patient admitted pursuant to this article shall be assigned a
6 patient advocate within twenty-four hours of such admission. Such
7 patient advocate may be an employee of the city or county in which the
8 hospital or facility is located; an employee or volunteer of a homeless
9 intervention services organization; a mental health expert; a person who
10 has experienced homelessness; or a family member of the patient.

11 § 10. Section 33.27 of the mental hygiene law is amended by adding a
12 new subdivision (d) to read as follows:

13 (d) Within twenty-four hours of admission to any facility operated or
14 licensed by the office of mental health a patient shall be informed of
15 their right to file a complaint with the ombudsman.

16 § 11. Section 33.27 of the mental hygiene law is amended by adding a
17 new subdivision (e) to read as follows:

18 (e) The office of the independent substance use disorder and mental
19 health ombudsman shall annually publish a report on involuntary admis-
20 sions that shall include the demographics of people admitted, the number
21 of complaints to the office, the due process for people admitted, the
22 nature of services provided after discharges, and the mental health and
23 housing stability outcomes of involuntary admissions.

24 § 12. The mental hygiene law is amended by adding a new section 22.13
25 to read as follows:

26 § 22.13 Medically recommended treatment.

27 Any addictive disorder services pursuant to this article shall include
28 medically recommended treatment by a person licensed to practice medi-
29 cine as set forth in article one hundred thirty-one of the education
30 law.

31 § 13. Section 29.07 of the mental hygiene law, subdivision (b) as
32 amended by chapter 37 of the laws of 2011, is amended to read as
33 follows:

34 § 29.07 Commissioner's powers over admissions to department facilities.

35 (a) ~~[The commissioner may by order defer admissions to]~~ If the commis-
36 sioner shall determine that overcrowding exists in any facility in the
37 department when the total number of patients therein exceeds its capaci-
38 ty to an extent which will not permit adequate care and treatment to be
39 provided patients, such commissioner shall authorize admission to anoth-
40 er facility with an appropriate operating certificate. The commissioner
41 may not defer admissions unless there is a state declaration of disaster
42 emergency pursuant to article two-B of the executive law.

43 (b) If the commissioner shall determine that overcrowding exists in
44 the department schools, ~~[he]~~ the commissioner may, within the amounts
45 appropriated therefor, authorize admission for care and treatment of any
46 person with a developmental disability to a designated facility approved
47 for such purposes by the commissioner. The patient and any liable rela-
48 tives shall be liable for payment of fees in accordance with article
49 forty-three of this chapter.

50 § 14. Subdivision (a) of section 9.47 of the mental hygiene law, as
51 amended by section 15 of chapter 351 of the laws of 2021, is amended to
52 read as follows:

53 (a) All directors of community services, health officers, and social
54 services officials, as defined by the social services law, are charged
55 with the duty of seeing that all persons with a mental illness within
56 their respective communities who are in need of or request medically

1 recommended care and treatment [~~at a hospital~~] are admitted to a hospi-
2 tal or other facility, or receive other services pursuant to the
3 provisions of this article or the social services law. Such services
4 shall include, but are not limited to, employment, health care, mental
5 health care, educational or vocational training, housing, and access to
6 supportive outpatient facilities such as clubhouses. Social services
7 officials and health officers shall notify the director of community
8 services of any such person coming to their attention. Pending the
9 determination of the condition of an alleged person with a mental
10 illness, it shall be the duty of the director of community services and,
11 if there be no such director, of the local health officer to provide for
12 the proper care of such person [~~in a suitable facility~~].

13 § 15. The opening paragraph of section 9.47 of the mental hygiene law,
14 as amended by section 16 of chapter 351 of the laws of 2021, is amended
15 to read as follows:

16 All directors of community services, health officers, and social
17 services officials, as defined by the social services law, are charged
18 with the duty of seeing that all persons with a mental illness within
19 their respective communities who are in need of or request medically
20 recommended care and treatment [~~at a hospital~~] are admitted to a hospi-
21 tal or other facility, or receive other services pursuant to the
22 provisions of this article or the social services law. Such services
23 shall include, but are not limited to, employment, health care, mental
24 health care, educational or vocational training, housing, and access to
25 supportive outpatient facilities such as clubhouses. Social services
26 officials and health officers shall notify the director of community
27 services of any such person coming to their attention. Pending the
28 determination of the condition of an alleged person with a mental
29 illness, it shall be the duty of the director of community services and,
30 if there be no such director, of the local health officer to provide for
31 the proper care of such person [~~in a suitable facility~~].

32 § 16. Subdivision (d) of section 9.37 of the mental hygiene law, as
33 amended by chapter 357 of the laws of 1991 and as relettered by chapter
34 343 of the laws of 1996, is amended to read as follows:

35 (d) After signing the application, the director of community services
36 or the director's designee shall be authorized and empowered to take
37 into custody, detain, transport, and provide temporary care for any such
38 person. Upon the written request of such director or the director's
39 designee it shall be the duty of a person or persons with expertise in
40 behavioral health and homeless intervention services to take into custo-
41 dy and transport any such person as requested and directed by such
42 director or designee. Such person or persons may request and shall
43 receive the accompaniment of peace officers, when acting pursuant to
44 their special duties, or police officers who are members of the state
45 police or of an authorized police department or force or of a sheriff's
46 department [~~to take into custody and transport any such person as~~
47 ~~requested and directed by such director or designee~~]. No officer shall
48 use coercive force to take a person into custody or pursue any such
49 person who flees unless directed by a person with expertise in behav-
50 ioral health, or homeless intervention services and, if so directed,
51 must refrain from any further coercion or pursuit at such direction;
52 provided, however, that this shall not apply to detaining a person
53 engaging in behavior likely to result in serious harm. Upon the written
54 request of such director or designee, an ambulance service, as defined
55 in subdivision two of section three thousand one of the public health
56 law, is authorized to transport any such person.

1 § 17. Intentionally omitted.

2 § 18. The mental hygiene law is amended by adding a new section 7.10
3 to read as follows:

4 § 7.10 Regulation and quality control of services for individuals with
5 severe mental illness.

6 This article sets forth provisions enabling the commissioner to regu-
7 late and assure the consistent high quality of treatment and services
8 provided within the state to its citizens with severe mental illness.
9 The commissioner shall evaluate mental health and behavioral health
10 outcomes, as well as issuance of treatment plans pursuant to section
11 29.13 of this chapter. The commissioner may adopt and promulgate any
12 regulation reasonably necessary to implement and effectively exercise
13 the powers and perform the duties conferred by this article. This arti-
14 cle shall govern the operation of programs, provision of services and
15 the facilities hereinafter described and the commissioner's powers and
16 authority with respect thereto.

17 § 19. Subdivision (n) of section 19.07 of the mental hygiene law, as
18 added by chapter 762 of the laws of 2022, is relettered subdivision (o)
19 and a new subdivision (p) is added to read as follows:

20 (p) The office shall not adopt any regulations on the dispensing of
21 opioid agonists, including but not limited to methadone, which would
22 result in additional limitations to access, continuing treatment,
23 dosing, and dispensing for home use, beyond those enumerated in federal
24 law and any waivers issued by the federal government.

25 § 20. Section 29.13 of the mental hygiene law, as added by chapter 332
26 of the laws of 1976, subdivision (b) as amended by chapter 135 of the
27 laws of 1993, is amended to read as follows:

28 § 29.13 Treatment plans.

29 (a) Subject to the regulations of the commissioner, the director of
30 each departmental facility shall require the development of a written
31 treatment plan to assure adequate ~~[care-and]~~ treatment for the health,
32 economic security, housing security, and welfare of each patient.

33 (b) The written treatment plan shall include, but not be limited to, a
34 statement of treatment goals; appropriate programs, treatment or thera-
35 pies to be undertaken to meet such goals; services pursuant to this
36 article and the social services law including, but not limited to,
37 employment, educational or vocational training, housing, and access to
38 supportive outpatient facilities such as clubhouses; and a specific
39 timetable for assessment of patient programs as well as for periodic
40 mental and physical reexaminations. In causing such a plan to be
41 prepared or when such a plan is to be revised, the following persons
42 shall be interviewed and provided an opportunity to actively participate
43 in such preparation or revision: the patient; ~~[an]~~ the patient advocate
44 or other authorized representative of the patient, to include the parent
45 or parents if the patient is a minor, unless such minor sixteen years of
46 age or older objects to the participation of the parent or parents and
47 there has been a clinical determination by a physician indicating that
48 the involvement of the parent or parents is not clinically appropriate
49 and such determination is documented in the record; upon the request of
50 the patient sixteen years of age or older, a significant individual to
51 the patient including any relative, close friend or individual otherwise
52 concerned with the welfare of the patient, other than an employee of the
53 facility.

54 (c) The director of each facility, in conjunction with a patient advo-
55 cate and any service provider for such discharged patient, shall monitor
56 the patient for at least thirty days to ensure compliance with the

patient treatment plan. If such patient is not in compliance, the local social services commissioner shall make a good faith effort to locate the patient and return them to treatment, and monitor such patient for at least an additional thirty days to ensure compliance.

(d) The commissioner may suspend, revoke, or limit the operating certificate of any facility found not to be providing medically recommended treatment plans pursuant to this section.

§ 21. Subdivision (k) of section 29.15 of the mental hygiene law, as amended by chapter 433 of the laws of 1976, is amended to read as follows:

(k) 1. No patient shall be discharged or conditionally released until such patient and the patient advocate approve the written treatment plan developed under section 29.13 of this article. No patient shall be required, as a condition precedent to [his] discharge, to agree to the terms of a written [service] treatment plan. If after the advisability of following the program proposed in the written [service] treatment plan has been explained to the patient who has been discharged or who is to be discharged, such patient expresses [his] their objection to such program or any part thereof, a notation of such objection shall be made in the patient's records.

2. A patient with a mental illness diagnosis may request and shall be entitled to receive inpatient or outpatient treatment recommended for such condition, irrespective of the ability of such person to pay. For the purposes of such treatment, a patient may be relocated to another facility having an appropriate operating certificate. In the absence of documentation of a mental illness diagnosis, attestation by the patient shall serve as presumptive evidence of such condition.

3. Upon discharge or conditional release, a patient shall be entitled to costs of transportation either to their place of usual residence, or another requested destination, if a social services official, in consultation with a patient advocate, determines that such person will have adequate support for their welfare.

§ 22. The mental hygiene law is amended by adding two new sections 9.34 and 9.36 to read as follows:

§ 9.34 Court authorization to continued inpatient treatment and care.

(a) If the director shall determine that a patient admitted upon an application supported by medical certification, for whom there is no court order authorizing retention for a specified period, is in need of discharge and if such patient or patient advocate does not agree with such discharge, the patient or anyone on their behalf shall apply to the supreme court or the county court in the county where the hospital is located for an order for continued inpatient treatment and care at the discretion of the patient. The hospital shall retain the patient for such further period. The court shall cause written notice of such application to be given to the director and a copy thereof shall be given personally or by mail to the persons required by this article to be served with notice of such patient's initial admission and to the mental hygiene legal service. Such notice shall state that a hearing may be requested and that failure to make such a request within five days, excluding Sunday and holidays, from the date that the notice was given to the director will permit the entry without a hearing of an order authorizing continued inpatient treatment and care.

(b) If no request is made for a hearing on behalf of the director within five days, excluding Sunday and holidays, from the date such notice of such application was given to such director, and if the mental hygiene legal service has not requested a hearing, the court receiving

1 the application may, if satisfied that the patient requires continued
2 inpatient care and treatment or transfer and continued retention, imme-
3 diately issue an order authorizing continued inpatient treatment and
4 care for such patient in such hospital.

5 (c) Upon the demand of the patient or of anyone on their behalf or
6 upon request of the mental hygiene legal service, the court shall, or
7 may on its own motion, fix a date for the hearing of the application, in
8 like manner as is provided for hearings in section 9.31 of this article.
9 The provisions of such section shall apply to the procedure for obtain-
10 ing and holding a hearing and to the granting or refusal to grant an
11 order of continued inpatient treatment and care by the court, except
12 that if the patient has already had a hearing, they shall not have the
13 right to designate initially the county in which the hearing shall be
14 held.

15 (d) If the director of a hospital, in which a patient is retained
16 pursuant to the foregoing subdivisions of this section, shall determine
17 that the condition of such patient requires discharge, such director
18 shall, if such patient does not agree to such discharge, apply during
19 the period of retention authorized by the last order of the court to the
20 supreme court or the county court in the county where the hospital is
21 located for an order authorizing discharge of such patient. The proce-
22 dures for obtaining any order pursuant to this subdivision shall be in
23 accordance with the provisions of the foregoing subdivisions of this
24 section; provided that the patient or anyone on such patient's behalf or
25 the mental hygiene legal service may request that the patient be brought
26 personally before the court.

27 (e) Nothing in this section shall prohibit the patient from voluntar-
28 ily agreeing to such discharge at any time; provided, however, that such
29 patient may not receive remuneration to agree to such discharge.

30 § 9.36 Review of court authorization to continued inpatient care and
31 treatment.

32 (a) If a hospital that has been ordered to continue provision of inpa-
33 tient treatment and care be dissatisfied with any such order, the direc-
34 tor may, during the period of authorized continued treatment and care,
35 obtain a rehearing and a review of the proceedings already had and of
36 such order upon a petition to a justice of the supreme court other than
37 the judge or justice presiding over the court making such order. Such
38 justice shall cause a jury to be summoned and shall try the question of
39 the mental illness and the need for retention of the patient so author-
40 ized to receive continued inpatient treatment and care. Any such direc-
41 tor applying for such review may waive the trial of the fact by a jury
42 and consent in writing to trial of such fact by the court. If the
43 verdict of the jury, or the decision of the court when jury trial has
44 been waived, be that such person does not have a mental illness or is
45 not in need of retention the justice shall forthwith discharge such
46 patient, but if the verdict of the jury, or the decision of the court
47 where a jury trial has been waived, be that such person is in need of
48 retention the justice shall certify that fact and make an order author-
49 izing continued inpatient treatment and care. Such order shall be
50 presented, at the time of authorization, to, and filed with, the direc-
51 tor, and a copy thereof shall be forwarded to the department by such
52 director and filed in the office thereof. Proceedings under the order
53 shall not be stayed pending an appeal therefrom, except upon an order of
54 a justice of the supreme court, made upon a notice and after a hearing,
55 with provisions made therein for such temporary care or confinement of
56 the alleged person with a mental illness as may be deemed necessary.

(b) Nothing in this section shall prohibit the patient from voluntarily agreeing to such discharge at any time; provided, however, that such patient may not receive remuneration to agree to such discharge.

§ 23. Section 29.19 of the mental hygiene law, as amended by chapter 408 of the laws of 1999, is amended to read as follows:

§ 29.19 Powers and duties [~~of peace officers acting pursuant to their special duties and police officers~~] to apprehend, restrain, and transport persons to facilities.

A person who has been committed or admitted to a department facility or a hospital licensed or operated by the office of mental health and who has been reported as escaped therefrom or from lawful custody, or who resists or evades lawful custody; and any patient for whom the director of a hospital operated by the office of mental health, or the director's designee, has terminated a conditional release and ordered such patient to return to such facility; and any patient for whom a director of an assisted outpatient treatment program, as defined in subdivision (a) of section 9.60 of this chapter, or the director's designee, or anyone designated pursuant to section 9.37 of this chapter, has directed the removal to a hospital pursuant to subdivision (n) of section 9.60 of this chapter, may be apprehended, restrained, transported to, and returned to such school or hospital by any person or persons with expertise in behavioral health and homeless intervention services, who may request the assistance of a peace officer, acting pursuant to [~~his~~] their special duties, or any police officer who is a member of an authorized police department or force or of a sheriff's department, and it shall be the duty of any such officer to assist any representative of a department or licensed facility, or an assisted outpatient treatment program, to take into custody any such person or patient upon the request of such representative, director or designee. No officer shall use coercive force to take a person into custody or pursue any such person who flees unless directed by a person with expertise in behavioral health, or homeless intervention services, and if so directed must refrain from any further coercion or pursuit at such direction; provided, however, that this shall not apply to detaining a person engaging in behavior likely to result in serious harm.

§ 24. Section 29.19 of the mental hygiene law, as amended by chapter 843 of the laws of 1980, is amended to read as follows:

§ 29.19 Powers and duties of peace officers acting pursuant to their special duties and police officers to apprehend, restrain, and transport persons to facilities.

A person who has been committed or admitted to a department facility and who has been reported as escaped therefrom or from lawful custody, or who resists or evades lawful custody, may be apprehended, restrained, transported to, and returned to such school or hospital by any person or persons with expertise in behavioral health and homeless intervention services, who may request the assistance of a peace officer, acting pursuant to [~~his~~] their special duties, or any police officer, and it shall be the duty of any such officer to assist any representative of a department facility to take into custody any such person upon the request of such representative. No officer shall use coercive force to take a person into custody or pursue any such person who flees unless directed by a person with expertise in behavioral health, or homeless intervention services, and if so directed must refrain from any further coercion or pursuit at such direction; provided, however, that this shall not apply to detaining a person engaging in behavior likely to result in serious harm.

§ 25. Subdivisions (c) and (e) of section 29.27 of the mental hygiene law, as amended by chapter 322 of the laws of 2021, are amended to read as follows:

(c) An incarcerated individual-patient may be retained for care and treatment in the facility designated by the commissioner for the period stated in the order committing the incarcerated individual-patient to the custody of the department unless sooner transferred or discharged in accordance with law. If the incarcerated individual-patient requires inpatient care and treatment for mental illness beyond such authorized period such person may request and shall receive medically recommended services, irrespective of ability to pay. If such person does not voluntarily accept treatment, the director of the facility where ~~[he or she is]~~ they are kept in custody shall apply for an order of retention or subsequent orders of retention in accordance with the procedures set forth in article nine of this chapter for the retention of patients. The provisions of this chapter applying to the rights of patients with respect to notices, hearings, judicial review, writ of habeas corpus, and the services of the mental hygiene legal service shall apply to incarcerated individual-patients except that in no case shall an incarcerated individual-patient be discharged or released from custody prior to the time that such incarcerated individual-patient has completed ~~[his or her]~~ their term of imprisonment or that ~~[his or her]~~ their release from custodial confinement in the correctional facility or jail from which ~~[he or she]~~ such individual was delivered to the department has been duly authorized.

(e) When the director of the facility in which the incarcerated individual-patient is in custody finds that the incarcerated individual-patient is no longer mentally ill or no longer requires hospitalization for care and treatment, ~~[he or she]~~ they shall so notify the incarcerated individual-patient and commissioner of corrections and community supervision or, in the case of an incarcerated individual-patient coming from a jail or correctional institution operated by local government, the officer in charge of the jail or correctional institution from which the incarcerated individual-patient was committed. The commissioner of corrections and community supervision or such officer, as the case may be, shall immediately arrange to take such incarcerated individual-patient into custody and return ~~[him or her]~~ them to a correctional facility or to the jail or correctional institution operated by local government. Upon return to the correctional institution, such incarcerated individual shall be eligible for review for merit termination of sentence and discharge from presumptive release, parole, conditional release and release to post-release supervision pursuant to section two hundred five of the correction law.

§ 26. Subdivision 1 of section 43 of the social services law, as amended by chapter 458 of the laws of 1986, is amended and a new subdivision 12 is added to read as follows:

1. ~~[Within the limits of funds available in the homeless housing and assistance fund, the]~~ The commissioner is hereby authorized to enter into contracts with municipalities to provide state financial assistance for the project costs attributable to the establishment of homeless housing projects. The municipalities that enter into contracts with the commissioner shall undertake the establishment of the homeless housing project or shall contract with a not-for-profit corporation or charitable organization to undertake the project, pursuant to this article.

12. No later than five years after the effective date of this subdivision, no municipality, not-for-profit corporation or subsidiary thereof,

1 public corporation or charitable organization or subsidiary thereof
2 shall operate, enter into or renew a contract for any homeless housing
3 project intended for occupancy more than thirty days other than a shel-
4 ter composed of single room occupancy units.

5 § 27. Subdivision 5 of section 45 of the social services law, as
6 amended by chapter 349 of the laws of 1994, is amended to read as
7 follows:

8 5. "Single room occupancy unit" shall mean a private room providing
9 living and sleeping space for no more than two persons or one family
10 with access to bathing and toilet facilities, within a building or
11 portion thereof which is operated by an eligible applicant[~~;~~ ~~provided,~~
12 ~~however, that in no event shall such unit be located in:~~

13 ~~(a) hotels, motels or other dwellings occupied transiently;~~
14 ~~(b) shelters for families or adults, as defined by the commissioner;~~
15 ~~(c) residential facilities or institutions which are required to be~~
16 ~~licensed by any state agency;~~
17 ~~(d) college or school dormitories;~~
18 ~~(e) clubhouses;~~
19 ~~(f) housing intended for use primarily or exclusively by the employees~~
20 ~~of a single company or institution; or~~
21 ~~(g) convents or monasteries].~~

22 The unit itself may contain a kitchen and/or a bathroom.

23 § 28. Subdivision 4 of section 81 of the social services law, as
24 amended by chapter 863 of the laws of 1977, is amended to read as
25 follows:

26 4. Such annual reports shall include an itemized statement of all
27 money received by the social services official and all money expended
28 ~~[by him]~~, and a detailed statement in regard to the recipients of public
29 assistance and care, and outcomes of homeless persons served that shall
30 include but is not limited to housing stability, behavioral health
31 outcomes, and employment. Town and city social services officers shall
32 furnish the county commissioner with all data, relating to their work
33 and persons in receipt of public assistance and care, necessary to
34 enable the county commissioner to make the reports required by the
35 department.

36 § 29. Section 71-a of the correction law, as amended by chapter 322 of
37 the laws of 2021, is amended to read as follows:

38 § 71-a. Transitional accountability plan. Upon admission of an incar-
39 cerated individual committed to the custody of the department under an
40 indeterminate or determinate sentence of imprisonment, the department
41 shall develop a transitional accountability plan. Such plan shall be a
42 comprehensive, dynamic and individualized case management plan based on
43 the programming and treatment needs of the incarcerated individual. The
44 purpose of such plan shall be to promote the rehabilitation of the
45 incarcerated individual and their successful and productive reentry and
46 reintegration into society upon release. To that end, such plan shall be
47 used to prioritize programming and treatment services for the incarcer-
48 ated individual during incarceration and any period of community super-
49 vision. The commissioner ~~[may]~~ shall consult with the office of mental
50 health, the office of ~~[alcoholism and substance abuse]~~ addiction
51 services and supports, the board of parole, the department of health,
52 and other appropriate agencies in the development of transitional case
53 management plans.

54 § 30. Paragraph (c) of subdivision 6 of section 72-a of the correction
55 law is REPEALED.

§ 31. Section 5 of chapter 554 of the laws of 1986, amending the correction law and the penal law relating to providing for community treatment facilities and establishing the crime of absconding from the community treatment facility, as amended by section 22 of part A of chapter 55 of the laws of 2023, is amended to read as follows:

§ 5. This act shall take effect immediately and shall remain in full force and effect until September 1, ~~2025~~ 2027, and provided further that the commissioner of correctional services shall report each January first and July first during such time as this legislation is in effect, to the ~~chairmen~~ chair of the senate crime victims, crime and correction committee, the senate codes committee, the assembly correction committee, and the assembly codes committee, the number of individuals who are released to community treatment facilities during the previous six-month period, including the total number for each date at each facility who are not residing within the facility, but who are required to report to the facility on a daily or less frequent basis.

§ 32. Subdivisions 5 and 15 of section 201 of the correction law, subdivision 5 as amended by chapter 484 of the laws of 2022, subdivision 15 as added by section 32 of subpart A of part C of chapter 62 of the laws of 2011, are amended to read as follows:

5. The department shall assist incarcerated individuals eligible for community supervision and individuals who are on community supervision to secure employment, educational or vocational training, ~~and~~ housing and treatment pursuant to the mental hygiene law. Any program the department requires a person on community supervision to take as a condition of such supervision shall not unreasonably interfere with such person's employment, educational or vocational training schedule unless such program is a residential treatment program.

15. The commissioner shall provide an annual report to the temporary president of the senate, the speaker of the assembly, the minority leader of the senate and minority leader of the assembly, commencing January first, two thousand twelve. Such report shall include but not be limited to the number of persons: released to community supervision and the release type; supervised on community supervision during the preceding year; whose community supervision was revoked; returned to incarceration for conviction of a new felony committed while on community supervision; transferred out of state pursuant to the Interstate Compact for Adult Supervision. In addition, the commissioner shall provide information on behavioral health and housing outcomes, and other available information regarding community supervision to the temporary president of the senate, the speaker of the assembly, the minority leader of the senate and minority leader of the assembly upon request.

§ 33. Section 205 of the correction law is amended by adding a new subdivision 1-a to read as follows:

1-a. A merit termination may be granted if it is determined by the department that such person cannot be reasonably expected to reoffend if provided services pursuant to the social services law or the mental hygiene law, and such person is not on presumptive release, parole, conditional release or release to post-release supervision from a term of imprisonment imposed for any of the following offenses:

(a) murder in the first degree;

(b) unlawful imprisonment in the first degree, kidnapping in the first degree, or kidnapping in the second degree, in which the victim is less than seventeen years old and the offender is not the parent of the victim;

1 (c) an offense defined in article two hundred thirty of the penal law
2 involving the prostitution of a person less than nineteen years old; or
3 (d) an offense defined in article two hundred sixty-three of the penal
4 law.

5 § 34. Paragraph (c) of subdivision 1 of section 273 of the correction
6 law, as added by section 1 of part SS of chapter 56 of the laws of 2009,
7 is amended to read as follows:

8 (c) having verified community ties in one of the following areas:
9 employment, permanent residence or receipt of homeless intervention
10 services and family.

11 § 35. Subdivision 6 of section 400 of the correction law, as added by
12 chapter 766 of the laws of 1976, is amended to read as follows:

13 (6) "Mental illness" means an affliction with a mental disease or
14 mental condition which is manifested by a disorder or disturbance in
15 behavior, feeling, thinking, or judgment to such an extent that the
16 person afflicted requires care and treatment; or an addictive disorder
17 as defined in the mental hygiene law.

18 § 36. Section 401 of the correction law is amended by adding a new
19 subdivision 1-a to read as follows:

20 1-a. A mental health clinician, or the highest ranking facility secu-
21 rity supervisor in consultation with a mental health clinician who has
22 interviewed the incarcerated individual, may determine that such incar-
23 cerated individual can receive therapeutic programming and/or mental
24 health treatment in an outpatient facility with an appropriate operating
25 certificate while living out-of-cell if such incarcerated person is
26 reasonably safe to be at-large. Such determination shall be documented
27 in writing.

28 § 37. Subdivision 6 of section 401 of the correction law, as separate-
29 ly amended by section 9 of part NNN of chapter 59 and chapter 322 of the
30 laws of 2021, is amended to read as follows:

31 6. The department shall ensure that the curriculum for new correction
32 officers, and other new department staff who will regularly work in
33 programs providing mental health treatment for incarcerated individuals,
34 shall include at least eight hours of training about the types and symp-
35 toms of mental illnesses, the goals of mental health treatment, the
36 prevention of suicide and training in how to effectively and safely
37 manage incarcerated individuals with mental illness. Such training may
38 be provided by the office of mental health or the justice center for the
39 protection of people with special needs. All department staff who are
40 transferring into a residential mental health treatment unit shall
41 receive a minimum of eight additional hours of such training, and eight
42 hours of annual training as long as they work in such a unit. All secu-
43 rity, program services, mental health and medical staff with direct
44 incarcerated individual contact shall receive training each year regard-
45 ing identification of, and care for, incarcerated individuals with
46 mental illnesses. The department shall provide additional training on
47 these topics on an ongoing basis as it deems appropriate. All staff
48 working in a residential mental health treatment unit shall also receive
49 the training mandated in paragraph (n) of subdivision six of section one
50 hundred thirty-seven of this chapter. All department staff shall have
51 the obligation to report signs of mental illness to a supervisor.

52 § 38. Subdivisions 1, 2, 3, 9 and 10 of section 402 of the correction
53 law, as amended by chapter 351, subdivisions 1, 2, 3 and 9 as separately
54 amended by chapter 322 of the laws of 2021, are amended and a new subdi-
55 vision 12-a is added to read as follows:

1 1. ~~[Whenever the]~~ The physician of any correctional facility, any
2 county penitentiary, county jail or workhouse, any reformatory for
3 women, or of any other correctional institution, shall ~~[report in writ-~~
4 ~~ing to the superintendent that]~~ conduct a personal examination and
5 review medical records of any person undergoing a sentence of imprison-
6 ment or adjudicated to be a youthful offender or juvenile delinquent
7 confined therein within five days of such person's incarceration, and
8 every twelve months thereafter. If the physician determines that the
9 incarcerated person has, in ~~[his or her]~~ such physician's opinion, a
10 mental illness, ~~[such superintendent shall apply to a judge of the coun-~~
11 ~~ty court or justice of the supreme court in the county to cause an exam-~~
12 ~~ination to be made of such person by two examining physicians. Such~~
13 ~~physicians shall be designated by the judge to whom the application is~~
14 ~~made.]~~ such physician shall cause an examination to be made of such
15 person by a second examining physician within twenty-four hours. Each
16 such physician, if satisfied, after a personal examination, that such
17 incarcerated individual has a mental illness and in need of care and
18 treatment, shall make a certificate to such effect. ~~[Before making such~~
19 ~~certificate, however, he or she shall consider alternative forms of care~~
20 ~~and treatment available during confinement in such correctional facili-~~
21 ~~ty, penitentiary, jail, reformatory or correctional institution that~~
22 ~~might be adequate to provide for such incarcerated individual's needs~~
23 ~~without requiring hospitalization.]~~ If the examining physician knows
24 that the person ~~[he or she is]~~ they are examining has been under prior
25 treatment, ~~[he or she]~~ such physician shall, insofar as possible,
26 consult with the physician or psychologist furnishing such prior treat-
27 ment prior to making ~~[his or her]~~ such certificate.

28 2. In the city of New York, ~~[if]~~ within five days of an individual's
29 incarceration and every twelve months thereafter, the physician of a
30 workhouse, city prison, jail, penitentiary or reformatory ~~[reports in~~
31 ~~writing to the superintendent of such institution that a prisoner~~
32 ~~confined therein, serving a sentence of imprisonment, in his or her~~
33 ~~opinion]~~ shall conduct a personal examination and review medical records
34 of such incarcerated individual. If, in the opinion of such physician,
35 the incarcerated individual has a mental illness, or if medical records
36 for such incarcerated individual document diagnosis of an ongoing seri-
37 ous mental illness, the superintendent of said institution shall either
38 transfer said ~~[prisoner]~~ incarcerated individual to Bellevue or Kings
39 county hospital for observation as to ~~[his or her]~~ such individual's
40 mental condition by two examining physicians or shall secure two examin-
41 ing physicians to make such examination and review medical records in
42 ~~[his]~~ the institution. Each such physician, if satisfied after a
43 personal examination ~~[and]~~ observation, and review of medical records
44 that the ~~[prisoner]~~ individual has a mental illness ~~[and]~~ in need of
45 care and treatment, shall make a certificate to such effect. ~~[Before~~
46 ~~making such certificate, however, he or she shall consider alternative~~
47 ~~forms of care and treatment available during confinement in such correc-~~
48 ~~tional facility, penitentiary, jail, reformatory or correctional insti-~~
49 ~~tution that might be adequate to provide for such incarcerated individ-~~
50 ~~ual's needs without requiring hospitalization.]~~ If the examining
51 physician knows that the person ~~[he or she is]~~ they are examining has
52 been under prior treatment, ~~[he or she]~~ they shall, insofar as possible,
53 consult with the physician or psychologist furnishing such prior treat-
54 ment prior to making ~~[his or her]~~ such certificate.

55 3. Upon such certificates of the examining physicians being so made,
56 it shall be delivered to the superintendent who, if the incarcerated

1 individual does not agree to voluntary admission, shall thereupon apply
2 by petition forthwith to a judge of the county court or justice of the
3 supreme court in the county, annexing such certificate to [~~his or her~~]
4 their petition, for an order committing such incarcerated individual to
5 a hospital for persons with a mental illness or outpatient facility with
6 an appropriate operating certificate. Upon every such application for
7 such an order of commitment, notice thereof in writing, of at least five
8 days, together with a copy of the petition, shall be served personally
9 upon the alleged person with a mental illness, and in addition thereto
10 such notice and a copy of the petition shall be served upon either the
11 [~~wife, the husband, the father or mother~~] spouse, parent or other near-
12 est relative of such alleged person with a mental illness, if there be
13 any such known relative within the state; and if not, such notice shall
14 be served upon any known friend of such alleged person with a mental
15 illness within the state. If there be no such known relative or friend
16 within the state, the giving of such notice shall be dispensed with, but
17 in such case the petition for the commitment shall recite the reasons
18 why service of such notice on a relative or friend of the alleged person
19 with a mental illness was dispensed with and, in such case, the order
20 for commitment shall recite why service of such a notice on a relative
21 or friend of the alleged person with a mental illness was dispensed
22 with. Copies of the notice, the petition and the certificates of the
23 examining physicians shall also be given the mental hygiene legal
24 service. The mental hygiene legal service shall inform the incarcerated
25 individual and, in proper cases, others interested in the incarcerated
26 individual's welfare, of the procedures for placement in a hospital or
27 outpatient facility having an appropriate operating certificate and of
28 the incarcerated individual's right to have a hearing, to have judicial
29 review with a right to a jury trial, to be represented by counsel and to
30 seek an independent medical opinion. The mental hygiene legal service
31 shall have personal access to such incarcerated individual for such
32 purposes.

33 9. Except as provided in subdivision two of this section pertaining to
34 [~~prisoners~~] incarcerated individuals confined in the city of New York,
35 an incarcerated individual of a correctional facility or a county jail
36 may be admitted on an emergency basis to the Central New York Psychiatric
37 Center upon the certification by two examining physicians, including
38 physicians employed by the office of mental health and associated with
39 the correctional facility in which such incarcerated individual is
40 confined, that the incarcerated individual suffers from a mental illness
41 which is likely to result in serious harm to [~~himself, herself~~] themselves
42 or others as defined in subdivision (a) of section 9.39 of the mental
43 hygiene law. Any person so committed shall be delivered by the superintendent
44 within a twenty-four hour period, to the director of the
45 appropriate hospital as designated in the rules and regulations of the
46 office of mental health. Upon delivery of such person to a hospital
47 operated by the office of mental health, if such person does not agree
48 to voluntary admission, a proceeding under this section shall immediately
49 be commenced.

50 10. If the director of a hospital for persons with a mental illness
51 shall deem that the condition of such person with a mental illness
52 requires [~~his~~] further retention in a hospital [~~he~~] the director shall,
53 during the period of retention authorized by the last order of the
54 court, apply to the supreme court or county court in the county where
55 such hospital is located, for an order authorizing continued retention
56 of such person with a mental illness. The procedures for obtaining any

1 order pursuant to this subdivision shall be in accordance with the
2 provisions of the mental hygiene law for the retention of involuntary
3 patients. A person may be discharged before the end of any sentence or
4 period of retention if the director determines it is medically appropri-
5 ate and such person is not likely to reoffend.

6 12-a. Prior to discharge, the facility director shall provide the
7 department with a treatment plan deemed medically appropriate and that
8 supports the housing stability and economic well-being of such person,
9 and the department, in consultation with the department of health must
10 approve such plan. If the director determines that no plan is needed,
11 they shall provide a written attestation to that effect.

12 § 39. Subdivision 4 of section 404 of the correction law, as amended
13 by chapter 322 of the laws of 2021, is amended to read as follows:

14 4. Every incarcerated individual who has received mental health treat-
15 ment pursuant to this article within three years of [~~his or her~~] their
16 anticipated release date from a state correctional facility shall be
17 provided with mental health discharge planning and, when necessary, [~~an~~
18 ~~appointment with a mental health professional~~] a course of treatment in
19 the community [~~who can prescribe~~] that can include prescription medica-
20 tions following discharge and sufficient mental health medications and
21 prescriptions to bridge the period between discharge and such time as
22 such mental health professional may assume care of the patient. Incar-
23 cerated individuals who have refused mental health treatment may also be
24 provided mental health discharge planning and any necessary appointment
25 with a mental health professional.

26 § 40. Subdivision 5 of section 201 of the correction law, as amended
27 by chapter 484 of the laws of 2022, is amended to read as follows:

28 5. The department shall assist incarcerated individuals eligible for
29 community supervision and individuals who are on community supervision
30 to secure employment, educational or vocational training, mental health
31 treatment, and housing. Any program the department requires a person on
32 community supervision to take as a condition of such supervision shall
33 not unreasonably interfere with such person's employment, educational or
34 vocational training schedule unless such program is a residential treat-
35 ment program.

36 § 41. The judiciary law is amended by adding a new article 5-C to read
37 as follows:

38 ARTICLE 5-C

39 MENTAL HEALTH COURTS

40 Section 178. Establishment of mental health courts.

41 178-a. Transfer of actions or proceedings to superior mental
42 health courts.

43 178-b. Transfer of actions or proceedings to local mental health
44 courts.

45 178-c. Procedure in a superior mental health court or local
46 mental health court upon transfer of actions or
47 proceedings thereto.

48 178-d. Reports.

49 § 178. Establishment of mental health courts. 1. Following consulta-
50 tion with the presiding justice of the appropriate appellate division,
51 the chief administrator of the courts shall establish mental health
52 courts in supreme court or county court ("superior mental health
53 courts") in any county and assign one or more justices or judges to
54 preside therein. Each superior mental health court shall have as its
55 purpose the hearing and determination of:

1 (a) criminal cases that are commenced in the superior court and that
2 are identified by the court as appropriate for disposition by a superior
3 mental health court; and

4 (b) criminal cases that are commenced in other courts of the county,
5 and that are identified as appropriate for disposition by a superior
6 mental health court and transferred to that court as provided for in
7 section one hundred seventy-eight-a of this article.

8 2. Where necessary to best utilize available court and community
9 resources for actions or proceedings involving defendants with mental
10 health problems, the chief administrator of the courts shall establish
11 mental health courts in one or more city or district courts or town or
12 village justice courts in such county, and assign one or more justices
13 or judges to preside therein. Each local mental health court shall have
14 as its purpose the hearing and determination of criminal actions or
15 proceedings that are commenced in a city or district court or town or
16 village justice court that are identified as appropriate for disposition
17 by a local mental health court and transferred to that court as provided
18 for in section one hundred seventy-eight-b of this article.

19 § 178-a. Transfer of actions or proceedings to superior mental health
20 courts. 1. (a) A local criminal court in a county in which a superior
21 mental health court has been established may, upon motion of the defend-
22 ant and with the consent of the district attorney, cause copies of
23 papers and other documents filed in such local criminal court in
24 connection with a criminal action or proceeding pending therein to be
25 sent to the superior mental health court:

26 (i) upon or after arraignment of the defendant on a local criminal
27 court accusatory instrument by which such action or proceeding was
28 commenced; or

29 (ii) upon or after commencement of a proceeding brought against the
30 defendant for the violation of a condition of a sentence of probation or
31 a sentence of conditional discharge.

32 (b) Not later than five days following receipt of the papers and other
33 documents, where there is a reasonable belief that a defendant has a
34 severe mental illness, the justice or judge presiding in the superior
35 mental health court shall review the medical records of such defendant
36 and shall cause a psychiatric evaluation of such defendant. If the
37 defendant is determined to have a severe mental illness, the justice or
38 judge presiding in the court shall order transfer, of the action or
39 proceeding to the superior mental health court, all originating papers
40 shall be sent from the originating court to the superior mental health
41 court, and all further proceedings shall be conducted therein. If the
42 justice or judge determines that a transfer of the action or proceeding
43 would not promote the administration of justice, they shall notify the
44 local criminal court from which the reference was received of such
45 determination, whereupon all further proceedings in such action or
46 proceeding shall be conducted in accordance with law.

47 2. (a) At any time while a criminal action or proceeding is pending in
48 a superior court in a county in which a superior mental health court has
49 been established, including a proceeding brought against defendant for
50 the violation of a condition of a sentence of probation or a sentence of
51 conditional discharge, a judge or justice of the court in which the
52 action or proceeding is pending may, upon motion of the defendant and
53 with the consent of the district attorney, cause copies of papers and
54 other documents filed in such court in connection with the action or
55 proceeding to be sent to the judge or justice presiding in the superior
56 mental health court for review of the appropriateness of the transfer.

1 (b) Not later than five business days following receipt of the papers
2 and other documents, the judge or justice presiding in the superior
3 mental health court shall determine whether or not a transfer of the
4 action or proceeding to the court would promote the administration of
5 justice. If such judge or justice determines that it would:

6 (i) such judge or justice, if sitting in supreme court, may order such
7 transfer, in which event the action or proceeding shall be referred for
8 disposition to the superior mental health court, all original papers
9 shall be sent to the superior mental health court, and all further
10 proceedings in such action or proceeding shall be conducted therein; or

11 (ii) such judge or justice, if sitting in county court, shall so noti-
12 fy the judge of the court who caused the papers and other documents to
13 be sent to them, and such justice may thereupon order such transfer, in
14 which event the action or proceeding shall be referred for disposition
15 to the superior mental health court, all original papers shall be sent
16 from the originating court to the superior mental health court, and all
17 further proceedings in such action or proceeding shall be conducted
18 therein. If the judge or justice presiding in the superior mental health
19 court determines that a transfer of the action or proceeding would not
20 promote the administration of justice, such judge or justice shall noti-
21 fy the originating court of such determination, whereupon all further
22 proceedings in such action or proceeding shall be conducted in accord-
23 ance with law.

24 3. Upon transfer of an action or proceeding to a mental health court,
25 a judge or justice, with the advice and consent of a psychiatrist and a
26 social services official, may order inpatient medical treatment, outpa-
27 tient treatment, or other medically recommended treatment, and may order
28 monitoring for compliance. Failure to comply with any such order may
29 result in a new hearing. Failure to comply with any such order shall not
30 be grounds for incarceration, probation, or fines.

31 4. Upon transfer of an action or proceeding to a mental health court,
32 the defendant shall be notified of social services available to them.

33 § 178-b. Transfer of actions or proceedings to local mental health
34 courts. 1. A local criminal court in a county in which a local mental
35 health court has been established may, upon motion of the defendant and
36 with the consent of the district attorney, cause copies of papers and
37 other documents filed in such local criminal court in connection with a
38 criminal action or proceeding therein to be sent to the local mental
39 health court:

40 (a) upon or after arraignment of a defendant on a local criminal court
41 accusatory instrument by which such action or proceeding was commenced;
42 or

43 (b) upon or after commencement of a proceeding brought against a
44 defendant for the violation of a condition of a sentence of probation or
45 a sentence of conditional discharge.

46 2. Not later than five days following receipt of the papers and other
47 documents, the justice or judge presiding in the local mental health
48 court, in consultation with the justice or judge in the court of origin,
49 shall review the medical records of such defendant and shall cause a
50 psychiatric evaluation of such defendant. If the defendant is determined
51 to have a severe mental illness, the justice or judge presiding in the
52 court shall order transfer, of the action or proceeding shall be trans-
53 ferred to the local mental health court, all originating papers shall
54 then be sent from the court of origin to the local mental health court,
55 and all further proceedings shall be conducted therein. If the presiding
56 justice or judge in the local mental health court or the justice or

1 judge presiding in the court of origin determines that a transfer of the
2 action or proceeding would not promote the administration of justice,
3 the action or proceeding will not be transferred and all further
4 proceedings in such action or proceeding shall be conducted in accord-
5 ance with law.

6 3. Upon transfer of an action or proceeding to a mental health court,
7 a judge or justice, with the advice and consent of a psychiatrist and a
8 social services official, may order inpatient medical treatment, outpa-
9 tient treatment, or other medically recommended treatment, and may order
10 monitoring for compliance. Failure to comply with any such order may
11 result in a new hearing. Failure to comply with any such order shall not
12 be grounds for incarceration, probation, or fines.

13 4. Upon transfer of an action or proceeding to a mental health court,
14 the defendant shall be notified of social services available to them.
15 Such services shall include, but are not limited to, employment, health
16 care, mental health care, educational or vocational training, housing,
17 and access to supportive outpatient facilities such as clubhouses.

18 § 178-c. Procedure in a superior mental health court or local mental
19 health court upon transfer of actions or proceedings thereto. Each
20 action or proceeding transferred to a superior court and referred for
21 disposition to a superior mental health court thereof and each action
22 transferred to a local court and referred for disposition in a local
23 mental health court thereof shall be subject to the same substantive and
24 procedural law as would have applied had there been no transfer.

25 § 178-d. Reports. Every five years the office of court administration
26 shall produce a report on outcomes on defendants in mental health courts
27 which shall include, but not be limited to, subsequent arrests, behav-
28 ioral health outcomes, and housing stability of such defendants.

29 § 42. Section 730.30 of the criminal procedure law, subdivision 3 as
30 amended by chapter 629 of the laws of 1974, is amended to read as
31 follows:

32 § 730.30 Fitness to proceed; order of examination.

33 1. At any time after a defendant is arraigned upon an accusatory
34 instrument [~~other than a felony complaint and~~] before the imposition of
35 sentence, or at any time after a defendant is arraigned upon a felony
36 complaint [~~and before he is held for the action of the grand jury~~], if
37 the defendant produces records or other evidence of a medical diagnosis
38 of mental illness or mental disability, or it is otherwise the opinion
39 of the court that the defendant may be an incapacitated person, the
40 court [~~wherein the criminal action is pending must issue an order of~~
41 ~~examination when it is of the opinion that the defendant may be an inca-~~
42 ~~pacitated person~~] shall direct such defendant to the mental health court
43 as provided for in article five-C of the judiciary law. For purposes of
44 such determination, the court may issue an order of examination.

45 2. When the examination [~~reports~~] report submitted to the court [~~show~~]
46 shows that [~~each~~] the psychiatric examiner is of the opinion that the
47 defendant is not an incapacitated person, the court may, on its own
48 motion, conduct a hearing to determine the issue of capacity, and it
49 must conduct a hearing upon motion therefor by the defendant or by the
50 district attorney. If no motion for a hearing is made, the criminal
51 action against the defendant must proceed. If, following a hearing, the
52 court is satisfied that the defendant is not an incapacitated person,
53 the criminal action against [~~him~~] such defendant must proceed; if the
54 court is not so satisfied, it must issue a further order of examination
55 directing that the defendant be examined by a different psychiatric
56 [~~examiners~~] examiner designated by the director.

3. When the examination reports submitted to the court show that each psychiatric examiner is of the opinion that the defendant is an incapacitated person, the court ~~[may, on its own motion, conduct a hearing to determine the issue of capacity and it must conduct such hearing upon motion therefor by the defendant or by the district attorney]~~ shall direct such defendant to the mental health court wherein the criminal action is pending.

4. When the examination reports submitted to the court show that the psychiatric examiners are not unanimous in their opinion as to whether the defendant is or is not an incapacitated person, or when the examination reports submitted to the superior court show that the psychiatric examiners are not unanimous in their opinion as to whether the defendant is or is not ~~[a dangerous]~~ an incapacitated person who, with a course of medically recommended treatment, nonetheless has a high probability of engaging in behavior likely to result in serious harm, the court must conduct a hearing to determine the issue of capacity or ~~[dangerousness]~~ such likelihood.

§ 43. Subdivision 9 of section 730.10 of the criminal procedure law, as added by section 1 of part Q of chapter 56 of the laws of 2012, is amended and three new subdivisions 10, 11 and 12 are added to read as follows:

9. "Appropriate institution" means: (a) a hospital operated by the office of mental health or a developmental center operated by the office for people with developmental disabilities; ~~[or]~~ (b) a hospital licensed by the department of health which operates a psychiatric unit licensed by the office of mental health, as determined by the commissioner ~~[provided, however, that any]; or (c) an outpatient facility having an appropriate operating certificate. Any such hospital or outpatient facility~~ that is not operated by the state ~~[shall]~~ may qualify as an "appropriate institution" ~~[only pursuant to the terms of an agreement between the commissioner and the hospital]~~ upon the consent of the hospital or outpatient facility. Nothing in this article shall be construed as requiring a hospital or outpatient facility to consent to providing care and treatment to an incapacitated person ~~[at such hospital]~~ if another appropriate institution offering comparable care and treatment is available.

10. "Likely to result in serious harm" has the same meaning as defined in section 9.01 of the mental hygiene law.

11. "Mental illness" has the same meaning as defined in section 1.03 of the mental hygiene law.

12. "Mental disability" has the same meaning as defined in section 1.03 of the mental hygiene law.

§ 44. The public health law is amended by adding a new article 30-D to read as follows:

ARTICLE 30-D

EMERGENCY BEHAVIORAL HEALTH SERVICES

Section 3080. Description.

3081. Definitions.

3082. State emergency behavioral health services advisory committee.

§ 3080. Description. 1. Emergency behavioral health services is a system for the immediate recognition and management of sudden illness or behavior of someone (a) with mental illness or mental disability, (b) who is homeless, or (c) who is engaging in behavior that is the result of substance use disorder, and addresses emergency medical dispatch, prehospital emergency care, in-hospital emergency care, admission,

behavioral health services, homeless intervention services, and housing. Such response shall consist of a team including, but not limited to, members trained in emergency medical services, behavioral health services, addictive disorder services, and homeless intervention services.

2. For the purposes of satisfying this section, team members may include individuals who have experienced mental illness or homelessness. Such team shall not include a police officer or peace officer, provided, however, that such team may request one for accompaniment or intervention at any time.

§ 3081. Definitions. As used in this article:

1. "State emergency behavioral health services advisory committee" means the state emergency behavioral health services advisory committee provided for by this article.

2. "State emergency medical services council" means the state emergency medical services council established under article thirty of this chapter.

3. "State emergency medical advisory committee" means the state emergency medical advisory committee established under article thirty of this chapter.

4. "State trauma advisory committee" means the state trauma advisory committee established under article thirty-B of this chapter.

5. "Mental illness" has the same meaning as defined in section 1.03 of the mental hygiene law.

6. "Mental disability" has the same meaning as defined in section 1.03 of the mental hygiene law.

7. "Behavioral health services" has the same meaning as defined in section 1.03 of the mental hygiene law.

8. "Addictive disorder services" has the same meaning as defined in section 1.03 of the mental hygiene law.

9. "Homeless intervention services" means services rendered pursuant to section fifty of the social services law.

10. "Admission" refers to hospitalization of a person with mental illness pursuant to article nine of the mental hygiene law.

§ 3082. State emergency behavioral health services advisory committee.

1. There is hereby established in the department the state emergency behavioral health services advisory committee. It shall consist of behavioral health and homeless intervention advocates representative of all geographic areas of the state; of those occupations regularly involved in behavioral health services and homeless intervention services; and of those with experience receiving such services, appointed by the commissioner upon recommendation of appropriate statewide professionals or organizations, who shall serve for terms of four years, which may be renewed. It shall advise the department, the commissioner, the state emergency medical services council, the state emergency medical advisory committee, and the state trauma advisory committee regarding all aspects of emergency behavioral health services, including, but not limited to, emergency medical dispatch, prehospital emergency care, in-hospital emergency care, admission, behavioral health services, homeless intervention services, and housing. The state emergency medical services director, the state emergency medical services medical director, the state trauma medical director, the state trauma program manager, and the governor's highway traffic safety administrator, shall also serve as nonvoting ex-officio members.

2. The state emergency behavioral health services advisory committee shall meet as frequently as its business may require, but ordinarily no

1 less than quarterly. The members of the state emergency behavioral
2 health services advisory committee shall receive no compensation for
3 their services as members, but each shall be allowed the necessary and
4 actual expenses incurred in the performance of their duties under this
5 section.

6 3. The commissioner shall designate an officer or employee of the
7 department to assist the state emergency behavioral health services
8 advisory committee in the performance of its duties under this section,
9 to coordinate the activities of the state emergency behavioral health
10 services advisory committee and to facilitate communication between the
11 state emergency health services council, the state emergency medical
12 advisory committee, and the state trauma advisory committee.

13 4. In no event shall any member, officer, or employee of the state
14 emergency behavioral health services committee be liable for damages in
15 any civil action for any act done, failure to act, or statement or opin-
16 ion made, while discharging duties as a member, officer, or employee of
17 the state emergency behavioral health services advisory committee if
18 they shall have acted in good faith, with reasonable care.

19 § 45. Subdivision 1 of section 602 of the public health law is amended
20 by adding a new paragraph (g) to read as follows:

21 (g) Mental health services.

22 § 46. The administrative code of the city of New York is amended by
23 adding two new sections 21-335 and 21-336 to read as follows:

24 § 21-335 Mobile devices and post office boxes. 1. Every homeless
25 person, or one individual in a family which is identified as homeless
26 shall be entitled to a mobile phone capable of at least Short Message
27 Service (SMS) and electronic mail.

28 2. Every homeless person, or one individual in a family which is iden-
29 tified as homeless shall be entitled to a post office box or other mail-
30 ing address.

31 § 21-336 Shelter systems study. At least once every five years, the
32 commissioner shall undertake a dynamic study on needed improvements to
33 the shelter system. Such study shall be conducted by following the
34 attempts of at least five contracted persons, posing as homeless indi-
35 viduals, as they attempt to seek permanent housing and services. The
36 commissioner shall produce a report on those processes and make recom-
37 mendations for improvements.

38 § 47. Section 17-199.26 of the administrative code of the city of New
39 York, as added by local law number 108 of the city of New York for the
40 year 2023, and as renumbered by local law number 100 for the city of New
41 York for the year 2024, is amended to read as follows:

42 § 17-199.26 Mental health and behavioral health services outreach and
43 education. The department shall establish and implement an outreach and
44 education campaign to raise public awareness about programs that provide
45 low-cost and no-cost mental health services to New Yorkers who do not
46 qualify for or cannot afford health insurance based on federal guide-
47 lines. Such outreach and education shall include, as applicable, an
48 explanation of how individuals may access such services, including, but
49 not limited to, through referrals from primary care providers. The mate-
50 rials for such outreach and education campaign shall be made available
51 in English and the designated citywide languages, as defined in section
52 23-1101. The department shall provide pamphlets and conspicuously
53 display information on the program in all city agency buildings,
54 schools, shelters, and at hospitals operated by the New York city health
55 and hospitals corporation.

§ 48. The administrative code of the city of New York is amended by adding a new section 21-304.1 to read as follows:

§ 21-304.1 Application; process. 1. To the maximum extent possible:

a. The commissioner shall develop a single application for all programs under this chapter, or, in the alternative, a process whereby the information provided by an applicant in a single application can be populated into other applications.

b. An application for services shall not be closed due to a missed appointment or other noncompliance.

c. An applicant shall be presumed eligible for services under this chapter and shall receive such services pending verification. If the applicant is subsequently deemed ineligible, the commissioner may provide alternative services. If the applicant is found to be in violation of any provisions of article one hundred fifty-eight of the penal law relating to the receipt of services under this chapter, the commissioner may discontinue services and recover civil damages pursuant to section one hundred forty-five-b of the social services law.

§ 49. Section 21-314 of the administrative code of the city of New York, as added by local law number 57 for the city of New York for the year 1998, and as renumbered by local law number 19 for the city of New York for the year 1999, is amended to read as follows:

§ 21-314 Case management services. ~~[The]~~ Within fourteen days of admission, the commissioner shall provide case management services to all persons assigned to stay at the department's facilities or the facilities of organizations contracting with the department who are either waiting for the department to determine their eligibility for shelter or are receiving such shelter. Such case management services shall include, but not be limited to, assistance obtaining (a) medical treatment, (b) federal, state and local government documents including, but not limited to, birth certificates, marriage licenses, and housing records, ~~[and]~~ (c) food, medicine and other necessary supplies, (d) permanent housing, and (e) outpatient services including clubhouses; and shall address issues such as domestic violence, child abuse and mental illness~~[, when needed]~~ that shall include transferring such persons to medically recommended treatment. To this end, an examining physician will perform a psychiatric evaluation and review medical records of each such person, and shall refer such person to medically recommended treatment.

§ 50. Paragraphs 1, 10 and 11 of subdivision b of section 21-332 of the administrative code of the city of New York, as added by local law number 62 of the city of New York for the year 2023, are amended and a new paragraph 12 is added to read as follows:

1. The right to shelter, which shall not be contingent upon a person undergoing addictive disorder services;

10. The requirement that a shelter comply with the environmental standards set forth in section 491.18 of title 18 of the New York codes, rules and regulations and section 900.18 of such title, as applicable; ~~[and]~~

11. The right to mental health treatment;

12. The right to a mobile phone and a post office box or other mailing address; and

13. Any other information the department deems appropriate.

§ 51. Severability. If any clause, sentence, paragraph, subdivision, section or part of this act shall be adjudged by any court of competent jurisdiction to be invalid, such judgment shall not affect, impair, or invalidate the remainder thereof, but shall be confined in its operation

1 to the clause, sentence, paragraph, subdivision, section or part thereof
2 directly involved in the controversy in which such judgment shall have
3 been rendered. It is hereby declared to be the intent of the legislature
4 that this act would have been enacted even if such invalid provisions
5 had not been included herein.

6 § 52. This act shall take effect on the first of January next succeed-
7 ing the date on which it shall have become a law; provided that:

8 (a) the amendments to section 9.48 of the mental hygiene law made by
9 section five of this act shall not affect the expiration and repeal of
10 such section and shall expire and be deemed repealed therewith;

11 (b) the amendments to subdivision (a) of section 9.47 of the mental
12 hygiene law made by section fourteen of this act shall be subject to the
13 expiration and reversion of such subdivision when upon such date the
14 provisions of section fifteen of this act shall take effect;

15 (c) the amendments to section 29.19 of the mental hygiene law made by
16 section twenty-three of this act shall be subject to the expiration and
17 reversion of such section when upon such date the provisions of section
18 twenty-four of this act shall take effect; and

19 (d) provided, however, that if local law number 100 of the city of New
20 York for the year 2024 shall not have taken effect on or before such
21 date then section forty-seven of this act shall take effect on the same
22 date and in the same manner as such local law of the laws of 2024 takes
23 effect.